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CT. [NAME]. [HOSPITAL] . Ultrasound , [HOSPITAL] imaging | [NAME] , [LOCATION], [LOCATION], 40 I

Automated Lab at some. location

Mr, [NAME]

62 yrs

Prof, Dr. [NAME]

13 [DATE]

Name:

Age:

Referred by:

Date of examination:

* Non-contrast Computed tomography of the chest and heart

[NAME]:

= Right upper lobar tiny nodular discrete with tr -in-bud configuration and
confluent opacities are noted mounting to consolidative patch.... Likely

reflecting infective pulmonary process... For clinico-lab correlation and

follow up.

Bilateral upper lobar apical fibrotic changes noted; likely sequel to old

infective process.

Bilateral lower lobar posterior basal atelectatic plates also seen.

Multiple tiny subcentimetric lucent lesions seen involving the dorsal spines.

Expansible lucent lesion is noted involving the anterior aspect of the right

8" rib. Other multiple tiny lesions are seen also involving other ribs and

sternum..... Features are in favor of diffuse infiltrative marrow process;

possibility of multiple myeloma considering patient's age is considered...

For clinico-laboratory correlation.

* Normal appearance of the tracheobronchial tree.

No evidence of hilar or mediastinal mass lesions or lymphadenopathy.

* No evidence of pleural collections or thickening.

= [NAME] cardiac size and shape, with no pericardial effusion.

Normal size and shape of the aorta and pulmonary trunk.

* [LOCATION] anterior aspect of the right 6" rib... Normal variant.

Prof. Dr. [NAME]

Prof. Dr. [NAME]

Dr. [NAME], [LOCATION]

Dr. [NAME], [LOCATION]. FRCKR

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